SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Nivolumab · every 2 weeks · adjuvant melanoma

Completely resected melanoma with lymph-node involvement or metastatic disease; the team confirms the stage III or resected stage IV indication. Uveal melanoma is excluded from this adjuvant pathway. · NCCP 00483 · version 13a · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Nivolumab helps immune cells act against melanoma. After complete surgery, its aim is to lower the risk of recurrence; this does not guarantee that melanoma will not return.

Schedule
Nivolumab every 2 weeks. Adjuvant treatment continues for up to 12 months, subject to safety review.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab

PD-1 CHECKPOINT IMMUNOTHERAPY

- ☐ Immune inflammation of bowel, lungs, liver or kidneys
- ☐ Thyroid, pituitary, adrenal and glucose problems
- ☐ Skin reactions and rare severe skin injury
- ☐ Rare heart, muscle, neurological and eye inflammation
- ☐ Infusion or injection reactions
- ☐ Some immune effects need long-term treatment

Discuss the exact diagnosis, treatment intent, expected benefits and uncertainty, suitable alternatives, the regimen-specific risks and monitoring, fertility and pregnancy precautions, and practical burdens. Confirm the treatment phase and route. This guide supports the conversation and does not replace individual consent.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00483 v13a.

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